

Chart 1

LOS: 5

OE ORDER

Patient Status: IP: Admit to Inpatient

Medical Reason: NSTEMI/SEPSIS ON ARRIVAL/COPD EXACERBATION

ED PHYSICIAN RECORD

Chief Complaint (Dyspnea)

Free Text HPI Notes:

Pt. with pmhx of COPD, presenting with dyspnea, 2 days, gradually worsening. Shortness of breath/dyspnea is now severe, constant, worse with any walking or talking, better slightly with medications. Pt. brought in by ambulance.pt. notes no chest pain, but has had diaphoresis and "pressure over my whole stomach and back" for 2 days. Pt. denies fever/chills. Pt. denies any hx of inhalation exposure/injury. No allergic reactions/swelling. No trauma to the airway or chest, or history of pneumothorax. Pt. denies chest pain/nausea/vomiting. Pt. denies any hemoptysis.

Pt. confirms no chest pain, notes cabgx4

Review of Systems:

All systems reviewed and Negative, except as noted.

Skin: Denies: burns, Bulle

ENT: Denies: ear discharge, nose bleeds

Respiratory: Denies: hemoptysis, pleuritic pain

Cardiovascular: Denies: cyanosis

Gastrointestinal: Denies: hematochezia, rectal pain

Genitourinary: Denies: decreased urination, purulent discharge

Neuro: DENIES: Word finding difficulty, Focal weakness

Additional Past Medical History: htn, COPD, MI

Additional Past Surgical History: CABGx4

Additional Family History: None

Additional Social History: - Etoh, + tob, - Drugs

Past Medical History - Adult:

Stated Complaint CARDIAC ALERT

Allergies:

Coded Allergies: No Known Allergies

Home Medications:

Active Scripts:

[Iprat-Airbuterol 0.5-3 Mg/3 MI] 3 ML INH RTQ4H

[Furosemide] 40 MG PO DAILY

[Klor-Con M20] 40 MEQ PO DAILY

[Hydromorphone Hcl] 2 MG PO Q4H PRN PRN

[Hydromorphone Hcl] 2 MG PO Q4H PRN PRN #30

Prov: Durkin,Melonie L 09/24/13

[Metoprolol Tartrate] 25 MG PO BID 6A 6P

[Metoprolol Tartrate] 25 MG PO BID 6A 6P #60 Ref 1

Prov: Durkin,Melonie L 09/24/13

Famotidine (Pepcid) 20 MG PO BEDTIME

Famotidine (Pepcid) 20 MG PO BEDTIME #14 TAB

Prov: Durkin,Melonie L 09/24/13

Lisinopril (Zestril) 10 MG PO BID

Ref 1

Prov: Durkin,Melonie L 09/24/13

Reported Medications:

Pravastatin (Pravachol) 40 MG PO DAILY

Aspirin EC (Ecotrin) 81 MG PO DAILY

Smoking status for patients 13 years old or older:

Current every day smoker

Vital Signs:

First Documented:

Result Date Time

O2 Delivery Non rebreather mask 10/13 0603

O2 Flow Rate 15 10/13 0603

Pulse Ox 98 10/13 0610

B/P 208/112 10/13 0610

B/P Mean 144 10/13 0610

Temp 36.7 10/13 0610

Pulse 146 10/13 0610

Resp 30 10/13 0610

FiO2 50 10/13 0655

Laboratory Tests:

10/13 10/13 10/13

0753 0609 0608

Chemistry

Sodium (135 - 145 mmol/L) 139

Potassium (3.5 - 5.2 mmol/L) 3.0 L

Chloride (95 - 110 mmol/L) 97

Carbon Dioxide (19 - 34 mmol/L) 34

BUN (6 - 22 mg/dL) 20

Creatinine (0.43 - 1.13 mg/dL) 0.65

Est GFR (CKD-EPI 2021) (≥ 90) > 90

Glucose (70 - 110 mg/dL) 196 H

Lactic Acid (0.4 - 2.0 mmol/L) 4.1 CH

Lactic Ac Sepsis Rflx (0.4 - 2.0 mmol/L) 2.9 H

Calcium (8.4 - 10.2 mg/dL) 9.0
Magnesium (1.6 - 2.4 mg/dL) 1.8
Total Bilirubin (0.1 - 1.2 mg/dL) 0.5
Conjugated Bilirubin (0.0 - 0.3 mg/dL) 0.0
Unconjugated Bilirubin (0.0 - 1.1 mg/dL) 0.2
AST (10 - 40 U/L) 61 H
ALT (10 - 60 U/L) 45
Total Alk Phosphatase (20 - 130 U/L) 49
Troponin I (0.000 - 0.034 ng/mL) 25.900 CH
Total Protein (5.5 - 8.7 g/dL) 7.3
Albumin (3.2 - 5.0 g/dL) 3.8

Hematology

WBC (4.0 - 10.5 10³/u) 15.7 H
RBC (3.93 - 5.22 10⁶/uL) 4.45
Hgb (11.2 - 15.7 G/DL) 13.7
Hct (34.1 - 44.9 %) 40.8
MCV (79.4 - 94.8 fL) 91.7
MCH (25.6 - 32.2 pg) 30.8
MCHC (32.2 - 35.5 g/dL) 33.6
RDW (11.7 - 14.4 %) 12.4
Plt Count (150 - 400 10³/uL) 292
MPV (9.4 - 12.3 fL) 10.4
Nucleated RBC % (auto) (0.0 - 0.2 %) 0.0
Immature Gran % (0.0 - 0.4 %) 0.8 H
Immature Gran # (0.00 - 0.03 10³/uL) 0.12 H
Neutrophils # (1.56 - 6.13 10³/uL) 9.58 H
Segmented Neutrophils (34.0 - 71.1 %) 60.9

Lymphocytes (19.3 - 51.7 %) 24.7
Lymphocytes # (1.18 - 3.74 10³/uL) 3.89 H
Monocytes (4.7 - 12.5 %) 12.6 H
Monocytes # (0.24 - 0.63 10³/uL) 1.98 H
Eosinophils (0.7 - 5.8 %) 0.7
Eosinophils # (0.04 - 0.36 10³/uL) 0.11
Basophils (0.1 - 1.2 %) 0.3
Basophils # (0.01 - 0.08 10³/uL) 0.04
Nucleated RBCs # (0.00 - 0.18 10³/uL) 0.00

10/13

0608

Blood Gas

Specimen Type Blood Venous

Puncture Site Other

pH (7.350 - 7.450) 7.308 L

pCO₂ (35 - 45 mm/Hg) 51 H

pO₂ (80.0 - 100.0 mm/Hg) 46.1 CL

HCO₃ (22.0 - 26.0 mmol/L) 25.1

Base Excess (-2.0 - 2.0 mmol/L) -1.8

O₂ Sat Preoxy (92.0 - 98.5 %) 66.1 L

ABG PO₂/FiO₂ Ratio (> 200) 46.1 L

ABG Carboxyhemoglobin (0.0 - 2.0 % THGB) 3.1 H

Hemoglobin (12.0 - 18.0 g/dL) 14.1

Hematocrit (37 - 52 %) 41

Glucose (70.0 - 110.0 mg/dL) 198.0 H

O₂ Delivery Device Non-Rebreather Mask

FiO2 (%) 100

Instrument 56207

BG Specimen Comment RCU7728

Chemistry

Sodium (135 - 145 mmol/L) 136.6

Potassium (3.5 - 5.2 mmol/L) 3.18 L

Chloride (95 - 110 mmol/L) 99

Lactic Acid (0.4 - 2.0 mmol/L) 4.0 H

Ionized Calcium (1.09 - 1.30 mmol/L) 1.17

Microbiology:

Date/Time Procedure - Status

Source Growth

10/13 0608 Blood Culture – RECD Blood

10/13 0608 Blood Culture – RECD Blood

IMPRESSION:

1. No pulmonary embolism or aortic dissection.
2. Cardiomegaly with left ventricular dilatation.
3. Interstitial edema and small bilateral pleural effusions.
4. Reflux of contrast into the hepatic veins suggestive of elevated right-sided pressures.
5. Coronary artery disease.

MEDICAL DECISION MAKING:

Pt. presents with workup concerning for COPD exacerbation, treated with bipap/solmedrol/ipratroprium/albuterol, and IV Magnesium, with significant improvement in condition, namely with sats improved to 98%, HR dropped from 140's to 80's. Pt. feels much more comfortable and relaxed. Her presentation initially concerning for possible STEMI, with unclear EKG, Cardiology consulted and reads no acute STEMI, but serial ekg's including posterior done. While concerning, they show evidence of right heart strain and chronic changes. In the clinical setting of 2 day hx of symptoms, with no active chest pain, deemed to be poor candidate for emergent cath. Trop returned over 25, consistent with missed/subacute MI. CTA pending to rule

out PE given her predominant symptom is dyspnea, although with trop of 25, a subacute MI 2 days ago seems more likely. CTA returned negative for PE.

Pt. has been given asa PTA, heparin bolus+GTT added here. Discussed case repeatedly with Dr. O, who is in agreement/guiding case from a cardiac standpoint.

Pt. had serial evaluations, monitoring for evidence of significant hypotension or clinical instability. Pt. was monitored closely, with repeat EKGs for any change in pain, serial troponins also sent. Pt. is clinically stable and is to be admitted for cardiac evaluation.

Clinical Impression:

Primary Impression: COPD exacerbation

Secondary Impressions: Acute respiratory disease, Myocardial infarct

H&P

Chief complaint: Chest and back pain

HPI:

This 64-year-old female with a past medical history of hypertension, hyperlipidemia, CHF, conscious CAD, COPD, diabetes type 2, and GERD presents to HCA L Hospital ER with back pain and chest pain that started 2 days ago but has since subsided. Patient was brought in by fire rescue as a cardiac alert due to ST elevation in AVF and lead III and depression in V2 and V3. ER physician spoke to STEMI on-call, Dr. O who said he did not feel that this was a current MI that needed to go to the cath lab today. Troponin 25.9. Potassium 3.0. WBCS 15.7, lactic acid 4.0. ABG shows a pH is 7.308, CO2 of 51, PO2 of 46.1. Patient placed on BiPAP in ER. Blood glucose 198. On assessment in ER room 14 family at bedside is very concerned. Auscultate wheezing throughout lung fields. Patient denies any chest pain, pressure, tightness at this time.

Spoke with ER physician agree admission for sepsis on admission/sub acute STEMI, COPD exacerbation.

Past medical history:

Reports: Congestive heart failure, COPD, Diabetes mellitus, GERD/gastritis, Hypertension, Dyslipidemia.

Smoking status for patients 13 years old or older: Current every day smoker

Allergies:

Coded Allergies: No Known Allergies

Diagnosis, Assessment Plan:

Problem List/A P:

1. COPD exacerbation
2. Acute respiratory disease
3. Myocardial infarct
4. CAD (coronary artery disease)
5. Diabetes type 2, uncontrolled
6. CHF (congestive heart failure)
7. Hyperlipidemia
8. Hypertension
9. Hypokalemia

Assessment Plan:

1. Subacute STEMI/NSTEMI. ST elevation in AVF and lead III and depression in V2 and V3. ER physician spoke to STEMI on-call, Dr. Ooi who said he did not feel that this was a current MI that needed to go to the cath lab today. Troponin 25.9.
 - * Admit to CV step-down
 - * Consult cardiology Dr. Ooi
 - * Monitor trending troponins and CK
 - * Echocardiogram
 - * TSH/A1c
 - * Metoprolol 25 mg b.i.d.
 - * ASA 81 mg daily
 - * Heparin drip
 - * Morphine 2 mg IV p.r.n. chest pain
2. COPD exacerbation, ABG shows a pH is 7.308, CO2 of 51, PO2 of 46.1. Patient given Solu-Medrol 125 mg IV, magnesium sulfate 2 g IV, DuoNeb treatment, albuterol treatment and placed on BiPAP in ER.
 - * Solu-Medrol 40 mg b.i.d.

- * DuoNeb q.4 hours p.r.n. wheezing/shortness of breath

3. Sepsis secondary to possible pneumonia, WBCS 15.7, lactic acid 4.0.

- * Rocephin 1 g daily

- * LR at 75 mL/hour

4. Hypokalemia, potassium 3.0.

- * Potassium chloride 20 mEq IV X 2

- * Electrolyte protocol in place

5. Hypertension

- * Monitor blood pressure per unit protocol

- * Continue antihypertensives

6. Hyperlipidemia

- * Lipid panel in a.m.

- * Atorvastatin 40 mg daily

7. CHF, history of

- * Monitor daily weight

- * Daily chest x-ray

8. Diabetes

- * Accu-Chek AC and HS

- * Medium sliding scale insulin

- * Hypoglycemic protocol

CONSULTATION REPORT

Critical care consult note**Chief complaint:** Chest pain**HPI:**

64 year old female with history of HTN, hyperlipidemia, CHF, CAD, COPD, diabetes mellitus type 2, GERD, presented to LRMC ED with c/o chest pain and back pain that started 2 days ago. Per note, she was brought in as a cardiac alert due to ST elevation in AVF and lead III and depression in V2 and V3. ICC was consulted due to shortness of breath and c/o chest pain. Spoke with Dr G and Dr O on call for STEMI, and cardiology requested starting nitro drip and will consider cardiac cath tonight if patient is having refractory chest pain after starting nitroglycerin drip. She reported chest pain (5/10) with diaphoresis.

64 year old female with history of HTN, hyperlipidemia, CHF, CAD, COPD, diabetes mellitus type 2, GERD, presented to LRMC ED with c/o chest pain and back pain that started 2 days ago. Per note, she was brought in as a cardiac alert due to ST elevation in AVF and lead III and depression in V2 and V3. ICC was consulted due to shortness of breath and c/o chest pain. Spoke with Dr G and Dr O on call for STEMI, and cardiology requested starting nitro drip and will consider cardiac cath tonight if patient is having refractory chest pain after starting nitroglycerin drip. She reported chest pain (5/10) with diaphoresis.

Past medical history:

Reports: Congestive heart failure, COPD, Diabetes mellitus, GERD/gastritis, Hypertension, Dyslipidemia.

Smoking status for patients 13 years old or older:

Former Smoker

Date last smoked: 10/12/24

Packs per day: 1

Years smoked: 40

Pack years: 40

Allergies:

Coded Allergies: No Known Allergies

Patient Weight and BMI:

Weight (kg): 73.182 BMI: 27.7

General appearance:

alert, awake, oriented

Head/Eyes: atraumatic, clear cornea

ENT: moist mucosal membranes, normal dentition

Neck: full range of motion, non-tender

Cardiovascular: abnormal S1/S2, tachycardia, normal capillary refill

Respiratory: decreased breath sounds

Abdomen: soft, non-tender

Perianal: deferred

Extremities: moves all, normal capillary refill, normal range of motion

Musculoskeletal: normal inspection, painless range of motion, straight leg raise neg

Neuro/CNS: alert, oriented X 3, CNII-XII intact, normal speech

Considered stroke alert: no

Cranial nerves:

Normal: II, III, IV, V, VI, VII, VIII, IX, X, XI, XII.

Skin: dry, intact, normal color, normal temperature, no rash

Psychiatry: normal affect, normal judgment/insight, normal mood

Assessment plan:

Subacute STEMI

COPD exacerbation

Sepsis secondary to pneumonia

Hypokalemia

Acute on chronic CHF exacerbation (unknown EF)

Diabetes mellitus type 2

Plan:

Neurology: Monitor neuro status

CV: Patient reported chest pain, substernal (5/10). Cardiology requested starting nitroglycerin drip, serial troponin, lasix 40 mg IV daily and will consider cardiac cath tonight if refractory chest pain with nitroglycerin drip.

Continue heparin drip. 2D Echo pending. Metoprolol 25 mg PO BID, aspirin 81 mg

PO daily, morphine 2 mg IV as needed for chest pain.

RR: Currently on BIPAP, tolerated well. Solumedrol 40 mg IV BID + duoneb q4 prn for SOB/wheezing. GI: NPO

GU: continue foley management.

ID: Trend WBC, lactic acid. Continue rocephin 1 GM IV daily, add Azithromycin 500 mg IV daily.

Integumentary: No acute issues noted.

Endocrinology: Maintain euglycemia. Accu-check ac/hs with humalog medium scale.

Hematology: Monitor for bleeding.

Tobacco use/counseling:

tobacco user, cessation counsel <3 min

Plan:

Reviewed the findings and plan as documented by G;

* my personal evaluation is

#late presentation STEMI

#CAD s/p CABG x 3 in 2013

#Hypertension

#Acute on Chronic Hypoxic Respiratory Failure

#Asthma

- cardiology following, planning for cath lab this am

- cont heparin drip

- cont nitro gtt

- trend troponins

- check echo

- daily aspirin, atorvastatin

- bipap as needed

- solumedrol 40 Q12H; wean

- scheduled duonebs

Consultation report

REASON FOR CONSULTATION: Subacute STEMI.

HISTORY OF PRESENT ILLNESS:

The patient is a 64-year-old lady presented yesterday to L ER with back pain and chest pain. This started Tuesday, subsided on Wednesday, and since then, she has been not noticing worsening shortness of breath. She had ST elevation inferiorly and a STEMI code was called and this was canceled by Dr. O on 10/13. The patient since then has been noticing worsening shortness of breath. She was seen this afternoon. She is on BiPAP. Husband at the bedside.

PAST MEDICAL HISTORY:

Congestive heart failure, COPD, diabetes, GERD, gastritis, hypertension, dyslipidemia.

ALLERGIES: NOTHING KNOWN.

SURGICAL HISTORY:

Coronary artery disease, status post coronary artery bypass graft, multiple stenting.

IMPRESSION AND PLAN:

1. ST-elevation myocardial infarction with late presentation. The patient has no chest pain for more than 48 hours now. She has had some worsening shortness of breath since Wednesday as per the husband, which warranted presentation to the emergency room. She was seen this afternoon on the wards on BiPAP, comfortable, not in distress. The patient will require cardiac cath, once more optimized from pulmonary standpoint. Continue IV heparin as well as anti-platelet therapy. The patient remains hemodynamically stable. Echocardiogram pending.
2. Congestive heart failure exacerbation, mild. She is close to euvolemia clinically. Continue IV furosemide. Check I's and O's, daily weight, FiO2 requirements.
3. History of coronary artery disease and stenting with LIMA to LAD, 3 saphenous grafts to the diagonal, obtuse marginal, and PDA.
4. Diabetes.

Nephro consult note – brief

Past medical history:

Reports: Congestive heart failure, COPD, Diabetes mellitus, GERD/gastritis, Hypertension, Dyslipidemia

Consultation report

REASON FOR CONSULTATION: Acute kidney injury/oliguria.

HISTORY OF PRESENT ILLNESS:

This is a 64-year-old female with history of hyperlipidemia, hypertension, congestive heart failure, and COPD, who was admitted complaining of chest and back pain.

Subsequently, the patient went into respiratory distress, for which she is intubated and on mechanical ventilation.

Upon admission, BUN and creatinine were found to be within normal limits; however, throughout the hospitalization, renal function has deteriorated, for which a consultation has been requested. At the time of this examination, she is intubated on mechanical ventilation, unable to provide any information. Of note, the patient was also very hypotensive, for which she has required high doses of pressors.

PAST MEDICAL HISTORY:

Significant for diabetes mellitus, hypertension, COPD, congestive heart failure, hyperlipidemia, and GERD.

ASSESSMENT:

1. Oliguric acute kidney injury, likely acute tubular necrosis secondary to hypotension and high doses of pressors.
2. Respiratory failure.
3. Coronary artery disease.
4. Diabetes mellitus.

PROGRESS NOTES

Clinical note:

Patient is 64 year old female with past medical history of hypertension, hyperlipidemia, CHF with triple bypass surgery in 2013, CAD, COPD, diabetes type 2, and GERD who presented to HCA Ld Hospital ER with back pain and chest pain that started 2 days ago but has since subsided. Patient was brought in by fire rescue as a cardiac alert due to ST elevation in AVF and lead III and depression in V2 and V3. ER physician spoke to STEMI on-call, Dr. O who said he did not feel that this was a current MI that needed to go to the cath lab today. Troponin 25.9. Potassium 3.0. WBCS 15.7, lactic acid 4.0. ABG shows a pH is 7.308, CO₂ of 51, PO₂ of 46.1. Patient placed on BiPAP in ER and taken up to 3Tower. At 20:15 repeat ABG with acute hypoxic respiratory failure with pH 7.428, pCO₂ 36.7, pO₂ 56.9, HCO₃ 23.7. Blood glucose 214. On assessment at bedside, patient was initially on 13/8 on Bipap at FiO₂ 60%. She was working hard to breath, with decreased breath sounds and wheezing in bilateral lung fields using accessory muscles and hyperventilating.

Her saturations were in range of 93-97%. Inspiratory/expiratory time adjusted to 18/8. EKG with findings of acute ischemia. ICU was consulted and patient moved to CVICU for further management.

Hospitalist progress note

Free text DxA P notes:

Assessment Plan:

Subacute STEMI/NSTEMI. ST elevation in AVF and lead III and depression in V2 and V3. ER physician spoke to STEMI on-call, Dr. Ooi who said he did not feel that this was a current MI that needed to go to the cath lab today. Troponin 25.9.

- * Consult cardiology
- * Monitor trending troponins and CK
- * Echocardiogram
- * Metoprolol 25 mg b.i.d.
- * ASA 81 mg daily
- * Heparin drip

COPD exacerbation, ABG shows a pH is 7.308, CO₂ of 51, PO₂ of 46.1. Patient given Solu-Medrol 125 mg IV, magnesium sulfate 2 g IV, DuoNeb treatment, albuterol treatment and placed on BiPAP in ER.

- * CCT following
- * Solu-Medrol 40 mg b.i.d. Wean as per CCT recs
- * DuoNeb q.4 hours p.r.n. wheezing/shortness of breath

Sepsis secondary to possible pneumonia, WBCS 15.7, lactic acid 4.0.

- * Rocephin 1 g daily
- * LR at 75 mL/hour

Hypokalemia, potassium 3.0.

- * Potassium chloride 20 mEq IV X 2

- * Electrolyte protocol in place

Hypertension

- * Monitor blood pressure per unit protocol
- * Continue antihypertensives

Hyperlipidemia

- * Lipid panel in a.m.
- * Atorvastatin 40 mg daily

Diastolic HFpEF, compensated/stable

- * Monitor daily weight
- * Daily chest x-ray

Diabetes

- * Accu-Chek AC and HS
- * Medium sliding scale insulin
- * Hypoglycemic protocol

Critical care progress note

Chief complaint:

Chest pain

HPI:

64 year old female with history of HTN, hyperlipidemia, CHF, CAD, COPD, diabetes mellitus type 2, GERD, presented to LRMC ED with c/o chest pain and back pain that started 2 days ago. Per note, she was brought in as a cardiac alert due to ST elevation in AVF and lead III and depression in V2 and V3. ICC was consulted due to shortness of breath and c/o chest pain. Spoke with Dr G and Dr O on call for STEMI, and cardiology requested starting nitro drip and will consider cardiac cath tonight if patient is having refractory chest pain after starting nitroglycerin drip. She reported chest pain (5/10) with diaphoresis.

Comments:

Pt short of breath

endorses 40 pack yr smoking history, active

Free text A P:

Assessment plan:

Subacute STEMI

COPD exacerbation

Sepsis secondary to pneumonia

Hypokalemia

Acute on chronic CHF exacerbation (unknown EF)

Diabetes mellitus type 2

Plan:

Neurology: Monitor neuro status

CV: Monitor BP and aim for tight control and HR < 100. Continue heparin drip. 2D Echo pending. Metoprolol 25 mg PO BID, aspirin 81 mg PO daily, morphine 2 mg IV as needed for chest pain. Will plan for cath tomorrow.

RR: Currently on BIPAP, tolerated well. Solumedrol 40 mg IV BID + duoneb q4 prn for SOB/wheezing and ctx/azithro for COPD exacerbation. Switched albuterol to levalbuterol as she was becoming very tachycardic.

GI: NPO after midnight

GU: continue foley management.

ID: Trend WBC, lactic acid. Continue rocephin 1 GM IV daily, add Azithromycin 500 mg IV daily.

Integumentary: No acute issues noted.

Endocrinology: Maintain euglycemia. Accu-check ac/hs with humalog medium scale.

Hematology: Monitor for bleeding.

Cardiology progress note

Free Text Dx A P Notes:

IMPRESSION AND PLAN:

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2. Congestive heart failure exacerbation, mild. She is close to euvolemia clinically. Continue IV furosemide. Check I's and O's, daily weight, FiO2 requirements.
3. History of coronary artery disease and stenting with LIMA to LAD, 3 saphenous grafts to the diagonal, obtuse marginal, and PDA.
4. Diabetes.

10/14

STEMI with late presentation

- Continue IV heparin as well as anti-platelet therapy
- possible cath tomorrow if respiratory status improved

Echocardiogram pending.

CHF exacerbation, mild.

- Continue IV furosemide.
- Check I's and O's, daily weight, FiO2 requirements.

Hx of CAD, with stenting with LIMA to LAD, 3 saphenous grafts to the diagonal, obtuse marginal, and PDA HTN, cont antihypertensives and adjust as needed HLD, cont lipid-lowering therapy

COPD exacerbation, pt currently on Bipap, wean as appropriate Diabetes

POC discussed with patient and family at bedside, including possible cardiac cath tomorrow if patient's respiratory status improves

Clinical Note:**Note:**

Patient intubated this morning by critical care team. Patient seen at bedside. Remains on mech ventilation.

Appreciate specialists.

2DE LVEF 25-30%, diffuse hypokinesis of Apex and lateral wall

Further cardiac workup pending stability of resp function

Critical care progress note

Chief complaint:

Chest pain

HPI:

64 year old female with history of HTN, hyperlipidemia, CHF, CAD, COPD, diabetes mellitus type 2, GERD, presented to LRMH ED with c/o chest pain and back pain that started 2 days ago. Per note, she was brought in as a cardiac alert due to ST elevation in AVF and lead III and depression in V2 and V3. ICC was consulted due to shortness of breath and c/o chest pain. Spoke with Dr G and Dr O on call for STEMI, and cardiology requested starting nitro drip and will consider cardiac cath tonight if patient is having refractory chest pain after starting nitroglycerin drip. She reported chest pain (5/10) with diaphoresis.

Free text A P:

Assessment plan:

Subacute STEMI

COPD exacerbation

Sepsis secondary to pneumonia

Hypokalemia

Acute on chronic CHF exacerbation (unknown EF)

Diabetes mellitus type 2

Plan:

Neurology: Monitor neuro status

CV: Monitor BP and aim for tight control and HR < 100. Continue heparin drip. 2D Echo pending. Metoprolol 25 mg PO BID, aspirin 81 mg PO daily, morphine 2 mg IV as needed for chest pain. Will plan for cath tomorrow.

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Integumentary: No acute issues noted.

Endocrinology: Maintain euglycemia. Accu-check ac/hs with humalog medium scale.

Hematology: Monitor for bleeding.

Cardiology progress note

Free Text Dx A P Notes:

IMPRESSION AND PLAN:

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4. Diabetes.

10/16

STEMI with late presentation

s/p cardiac cath;

- Moderate-to-severe decrease in LV function, EF 30% to 35%.
- Three grafts are open, LIMA to LAD, vein graft to diagonal, vein graft to marginal.
- RCA is totally occluded with LAD to PDA grafts from the LIMA injection.
- continue with medical therapy

Acute hypoxic respiratory failure, requiring intubation.

Echo reviewed;

- Left ventricle is moderately dilated.
- Markedly reduced systolic left ventricular function.
- EF range is estimated at 25 % -30 %.
- There is moderate diffuse hypokinesis- APEX AND LATERAL WALL CONTRACTING BETTER.
- Mild -moderate mitral regurgitation.
- Mild tricuspid regurgitation

Severe ischemic cardiomyopathy, EF 30%, hold beta-blocker given hypotension.

Continue on dobutamine drip

HFrEF, CHF exacerbation, mild.

- Continue IV furosemide.
- Check I's and O's, daily weight, FiO2 requirements.

Hx of CAD, with stenting with LIMA to LAD, 3 saphenous grafts to the diagonal, obtuse marginal, and PDA.

HTN, cont antihypertensives and adjust as needed

HLD, cont lipid-lowering therapy

COPD exacerbation, exacerbation.

Diabetes mellitus

Patient went into V-tach arrest today. ACLS and cardioversion. Started on amiodarone bolus.

Clinical Note:

Note:

Remains under care of CCT

Seen at bedside -- Intubated/mechanically ventilated

Chart reviewed

Optimize pulmonary status

Cardiology following, eventual ischemic w/u pending pulmonary stability

Critical care progress note

Chief complaint: Chest pain

HPI:

64 year old female with history of HTN, hyperlipidemia, CHF, CAD, COPD, diabetes mellitus type 2, GERD, presented to LRMC ED with c/o chest pain and back pain that started 2 days ago. Per note, she was brought in as a cardiac alert due to ST elevation in AVF and lead III and depression in V2 and V3. ICC was consulted due to shortness of breath and c/o chest pain. Spoke with Dr G and Dr O on call for STEMI, and cardiology requested starting nitro drip and will consider cardiac cath tonight if patient is having refractory chest pain after starting nitroglycerin drip. She reported chest pain (5/10) with diaphoresis.

10/16: vtach requiring ACLS and cardioversion. Interval placement PA catheter. Zosyn.

Free text A P:

Assessment plan:

Subacute STEMI

COPD exacerbation

Sepsis secondary to pneumonia

Hypokalemia

Acute on chronic CHF exacerbation (unknown EF)

Diabetes mellitus type 2

Plan:

Neurology: Monitor neuro status

CV: Monitor BP and aim for tight control and HR < 100. Continue heparin drip. 2D Echo pending. Metoprolol 25 mg PO BID, aspirin 81 mg PO daily, morphine 2 mg IV as needed for chest pain. Will plan for cath tomorrow.

RR: Currently on BIPAP, tolerated well. Solumedrol 40 mg IV BID + duoneb q4 prn for SOB/wheezing and ctx/azithro for COPD exacerbation. Switched albuterol to levalbuterol as she was becoming very tachycardic.

GI: NPO after midnight

GU: continue foley management.

ID: Trend WBC, lactic acid. Continue rocephin 1 GM IV daily, add Azithromycin 500 mg IV daily.

Integumentary: No acute issues noted.

Endocrinology: Maintain euglycemia. Accu-check ac/hs with humalog medium scale.

Hematology: Monitor for bleeding.

Cardiology progress note

Free Text Dx A P Notes:

IMPRESSION AND PLAN:

1. ST-elevation myocardial infarction with late presentation. The patient has no chest pain for more than 48 hours now. She has had some worsening shortness of breath since Wednesday as per the husband, which warranted presentation to the emergency room. She was seen this afternoon on the wards on BiPAP, comfortable, not in distress. The patient will require cardiac cath, once more optimized from pulmonary standpoint. Continue IV heparin as well as anti-platelet therapy. The patient remains hemodynamically stable. Echocardiogram pending.
2. Congestive heart failure exacerbation, mild. She is close to euvolemia clinically. Continue IV furosemide. Check I's and O's, daily weight, FiO2 requirements.
3. History of coronary artery disease and stenting with LIMA to LAD, 3 saphenous grafts to the diagonal, obtuse marginal, and PDA.
4. Diabetes.

10/17

STEMI with late presentation

s/p cardiac cath;

- Moderate-to-severe decrease in LV function, EF 30% to 35%.
- Three grafts are open, LIMA to LAD, vein graft to diagonal, vein graft to marginal.
- RCA is totally occluded with LAD to PDA grafts from the LIMA injection.
- continue with medical therapy

Acute hypoxic respiratory failure, requiring intubation.

Echo reviewed;

- Left ventricle is moderately dilated.
- Markedly reduced systolic left ventricular function.
- EF range is estimated at 25 % -30 %.

- There is moderate diffuse hypokinesis- APEX AND LATERAL WALL CONTRACTING BETTER.
- Mild -moderate mitral regurgitation.
- Mild tricuspid regurgitation

Severe ischemic cardiomyopathy, EF 30%, hold beta-blocker given hypotension.

Continue on dobutamine drip

HFrEF, CHF exacerbation, mild.

- Continue IV furosemide.
- Check I's and O's, daily weight, FiO2 requirements.

Hx of CAD, with stenting with LIMA to LAD, 3 saphenous grafts to the diagonal, obtuse marginal, and PDA. History of HTN, pt is hypotensive on Levophed. HLD, cont lipid-lowering therapy. COPD exacerbation, exacerbation. Diabetes mellitus, primary managing.

10/16 V-tach arrest today. ACLS and cardioversion. Started on amiodarone bolus.

CKD with minimal urine output.

Still on Levophed, dobutamine was discontinued

Still requiring high PEEP on the ventilator.

Metolazone 10mg x1 dose

NSR on tele.

Critical care progress note

Chief complaint: Chest pain

HPI:

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10/16: vtach requiring ACLS and cardioversion. interval placement PA catheter. Zosyn.

10/17: limited response to diuretics. goals of care discussions with proxy and extended family. AKI worsening, likely cardiorenal. Shock liver, hyperammonemia. Ischemic cardiomyopathy, EF 20%.

Free text A P:

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Subacute STEMI

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Diabetes mellitus type 2

Limited response to diuretics. Goals of care discussions with proxy and extended family. AKI worsening, likely cardiorenal. Shock liver, hyperammonemia. Ischemic cardiomyopathy, EF 20%.Nephrology consult- may be too unstable for CVVHD. Progressive MSOF

ID: Trend WBC, lactic acid. zosyn for CAP

Integumentary: No acute issues noted.

Endocrinology: SSI for hyperglycemia.

Critical care progress note

Chief complaint: Chest pain

HPI:

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Cardiology progress note

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4. Diabetes.

10/18

Multiorgan failure

STEMI with late presentation

s/p cardiac cath;

- Moderate-to-severe decrease in LV function, EF 30% to 35%.
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- Mild -moderate mitral regurgitation.
- Mild tricuspid regurgitation

Severe ischemic cardiomyopathy, EF 30%, hold beta-blocker given hypotension.

Continue on dobutamine drip

HFrEF, CHF exacerbation, mild.

- Continue IV furosemide.
- Check I's and O's, daily weight, FiO2 requirements.

Severe MR, LV function improving. Will need to MitraClip at some point.

Hx of CAD, with stenting with LIMA to LAD, 3 saphenous grafts to the diagonal, obtuse marginal, and PDA. History of HTN, pt is hypotensive on Levophed. HLD, cont lipid-lowering therapy
COPD exacerbation, exacerbation.

Diabetes mellitus, primary managing.

10/16 V-Tach arrest today. ACLS and cardioversion. Started on amiodarone bolus. CKD with minimal urine output.

Still on Levophed

FiO2 at 60%, peep dropped to 10.

ST on monitor.

Worsening renal failure with minimal urine output.

DISCHARGE SUMMARY

General Information

Discharge date: 10/18/24

Discharge diagnosis:

chest pain NSTEMI

Hospital course:

Subacute STEMI/NSTEMI. ST elevation in AVF and lead III and depression in V2 and V3. ER physician spoke to STEMI on-call, Dr. Ooi who said he did not feel that this was a current MI that needed to go to the cath lab today. Troponin 25.9.

- o Consult cardiology
- o Monitor trending troponins and CK
- o Echocardiogram
- o Metoprolol 25 mg b.i.d.
- o ASA 81 mg daily
- o Heparin drip

COPD exacerbation, ABG shows a pH is 7.308, CO2 of 51, PO2 of 46.1. Patient given Solu-Medrol 125 mg IV, magnesium sulfate 2 g IV, DuoNeb treatment, albuterol treatment and placed on BiPAP in ER.

- o CCT following
- o Solu-Medrol 40 mg b.i.d. Wean as per CCT recs
- o DuoNeb q.4 hours p.r.n. wheezing/shortness of breath

Sepsis secondary to possible pneumonia, WBCS 15.7, lactic acid 4.0.

- o Rocephin 1 g daily
- o LR at 75 mL/hour

Hypokalemia, potassium 3.0.

- o Potassium chloride 20 mEq IV X 2
- o Electrolyte protocol in place

Hypertension

- o Monitor blood pressure per unit protocol
- o Continue antihypertensives

Hyperlipidemia

- o Lipid panel in a.m.

- o Atorvastatin 40 mg daily

Diastolic HFpEF, compensated/stable

- o Monitor daily weight
- o Daily chest x-ray

Diabetes

- o Accu-Chek AC and HS
- o Medium sliding scale insulin
- o Hypoglycemic protocol

Disposition: Wean off BIPAP as tolerated with anticipation of LHC once resp status optimized. d/w family and patient -- agreeable to POC.
d/w nurse

DVT prophylaxis as ordered

DIET: Diabetic/cardiac heart healthy

Full Code

Medications reviewed

QUERY

QUERY TEXT:

Condition General 360MD

Based on your clinical judgment, please clarify the condition that represent the clinical indicators listed below, e.g. Lactic acidosis, Metabolic acidosis, Respiratory acidosis, Mixed metabolic and respiratory acidosis, abnormal lab results without clinical significance, or other more appropriate diagnosis based on the clinical indicators listed below?

The patient's Clinical Indicators include:

Lactic acid 4, 2.9, 2.6, 1.8, 2.4, 1.6 (Labs 10-13)

pH 7.308, PCO2 51, PO2 46.1, HCO3 25.1 (ABG 10-13)

Sepsis secondary to possible pneumonia, WBCS 15.7, lactic acid 4.0.(Hospitalist HP 10-13)

Acute on Chronic Hypoxic Respiratory Failure (Critical Care Consult 10-13)

Lactated Ringers 1000 ML BAG - Dose: 1000.0 - Ordered

Route: IV (MARD 10-13)

Options provided:

- Respond - Create new note now
- Disagree - Not applicable/Not valid
- Disagree - Clinically unable to determine / Unknown

-- Assign to another provider

QUERY RESPONSE: Lactic acidosis

QUERY TEXT:

Stage CKD 360MD Based on your clinical judgment, can you further specify the stage of the chronic kidney disease located in [CKD-PN 10/18(s)]?

Pertinent lab findings are as follows:

[CREATININE (mg/dL) 2.57H 2.14H 1.05 1.25H 0.81 0.65 and/or eGFR 20L 25L 59L 48L 81L > 90(s)]

Stages of Kidney Damage according to the National Kidney

Foundation are defined as follows:

- Stage 1 Kidney damage with normal kidney function (GFR >90)
- Stage 2 Kidney damage with mildly decreased kidney function (GFR 60-89)
- Stage 3a Kidney damage with mildly to moderately decreased kidney function (GFR 45-59)
- Stage 3b Kidney damage with moderately to severely decreased kidney function (GFR 30-44)
- Stage 4 Kidney damage with severely decreased kidney function (GFR 15-29)
- Stage 5 Kidney failure (GFR <15 or on dialysis)
- End Stage Renal Disease (GFR <15 or on dialysis or transplant)

The patient's Clinical Indicators include:

CREATININE (mg/dL) 2.57H 2.14H 1.05 1.25H 0.81 0.65

eGFR 20L 25L 59L 48L 81L > 90

GLUCOSE (mg/dL) 222H 202H 188H

QUERY RESPONSE: The patient has stage 4 chronic kidney disease.

Post intubation report

Intubation note

Unit called to: CVICU

Indications: respiratory failure

Patient receiving supplemental oxygen: BiPAP

Vital signs pre-intubation:

Result Date Time

Pulse Ox 96 10/15 0319

FiO2 50 10/15 0319

Pulse 85 10/15 0319

B/P 104/69 10/15 0003

B/P Mean 82 10/15 0003

Resp 20 10/15 0003

Temp 36.4 10/14 2356

O2 Delivery BiPAP 10/14 2009

O2 Flow Rate 1 10/14 2009

Pre-oxygenation Yes

Rapid Sequence Induction (RSI): Yes

IV induction: Yes

Medications given: etomidate, rocuronium

Technique: glidescope

Laryngoscope Blade: lo pro S3

Number of attempts: 1

Bilateral breath sounds Yes

ETCO2 detector color change to yellow: Yes

ET tube size: 7.5

ET tube taped at the lip at: (cm) 24

Vital signs post-intubation:

BP: [120]

HR: [90] bpm

RR: [16] / min

SP02: [100] %

Mechanical ventilation initiated: Yes

Ventilator mode: A/C

Ventilator settings:

PRVC

VT 400

RR 16

PEEP 5

Complications observed: none

Chest X-ray to follow by Attending Physician: Yes

Critical care time: Minutes: 30

RT: Ventilator Flowsheet + 10/18/2024 - 06:42 PM	
-	- RT VENTILATOR FLOWSHEET - -
Ventilator flowsheet treatment:	Adult
Mechanical ventilation start date:	20241015
Mechanical ventilation start time:	5.22
Mechanical ventilation stop date:	20241018
Mechanical ventilation stop time:	15.17
-	- VENTILATOR PATIENT ASSESSMENT - -
Airway type:	Oral ET tube cuffed
Airway tube size:	7.5mm

INTERVENTIONAL CARDIOLOGY

Cardiac catheterization

PROCEDURES:

1. Left heart catheterization.
2. Coronary arteriography.
3. Left ventriculography.
4. Iliac angiography.
5. Graft angiography, both arterial and venous.

INDICATION:

Acute pulmonary edema, non ST elevation myocardial infarction. The patient on the ventilator.

DESCRIPTION OF PROCEDURE:

After obtaining informed consent, the patient was brought to the cath lab. Right groin was prepped and draped in usual fashion. Single-wall needle with 0.035 wire, a 6-French sheath was placed in right femoral artery. Over the wire, a 6-French multipurpose catheter graft angiography was performed and then left and right coronary angiography, left ventriculography performed. Then, internal mammary angiography was performed.

COMPLICATIONS: None.

FINDINGS:

1. Hemodynamic Data:

LV end-diastolic pressure 30 plus mmHg.

2. Left ventriculography. Anterior wall moves well, but inferior wall akinetic, overall ejection fraction 30% to 35%.

CORONARY ANGIOGRAPHY:

1. Left main is 100% occluded. RCA is proximally occluded with right to right collaterals.
2. Graft angiography. LIMA to LAD is widely patent, which gives collaterals to the RCA.
3. Saphenous vein graft to diagonal is widely patent. Saphenous vein graft to marginal is widely patent. Iliac artery is widely patent.

CONCLUSIONS:

1. Moderate-to-severe decrease in LV function, EF 30% to 35%.
2. Three grafts are open, LIMA to LAD, vein graft to diagonal, vein graft to marginal. RCA is totally occluded with LAD to PDA grafts from the LIMA injection.

ORDER – DNR

RESUS-20241018-0002

Hospice Care: Y:Yes (Y)

DNR: Do Not Resuscitate

CASE MANAGEMENT**Comments:**

Treasure Coast Hospice

Discharge Disposition: 51 - Hospice-Facility